DRAFT PRODUCT INFORMATION

Company Core Data Sheet — kelvaparib
Kelvaparib (KLV-3308)
Marnholt Oncology GmbH

STATUS OF THIS DOCUMENT

Kelvaparib is an investigational product and holds no marketing authorisation in any country. This document is the company core data sheet maintained during clinical development. It states the information the sponsor proposes for the product information at the time of a marketing application, and is the reference against which proposed labelling changes are assessed. Expectedness for expedited reporting is determined against the reference safety information, Section 7 of the Investigator's Brochure (filed as reference-safety-information-kelvaparib), the sole governing document for listedness.

1. NAME OF THE MEDICINAL PRODUCT

Kelvaparib 150 mg film-coated tablets.

2. QUALITATIVE AND QUANTITATIVE COMPOSITION

Each film-coated tablet contains 150 mg of kelvaparib. The recommended dose is administered as two tablets twice daily.

3. PHARMACEUTICAL FORM

Film-coated tablet for oral administration.

4. CLINICAL PARTICULARS

4.1 Therapeutic indications

Kelvaparib is proposed as maintenance monotherapy for adults with recurrent high-grade serous ovarian carcinoma after prior platinum-based chemotherapy. The proposed indication is supported by study KLV-7701, a randomised, placebo-controlled study of kelvaparib as maintenance therapy.

4.2 Posology and method of administration

The recommended dose is 300 mg orally twice daily on a continuous schedule. Treatment should continue until disease progression, unacceptable toxicity or withdrawal of consent. Tablets should be swallowed whole and may be taken with or without food.

Dose modification. For a grade 3 or higher haematological adverse reaction, treatment should be withheld until recovery to grade 1 or baseline and resumed at 225 mg twice daily. A second reduction to 150 mg twice daily is permitted. Treatment should be discontinued if a grade 3 or higher reaction recurs at the lowest dose, or where haematological recovery has not occurred within 28 days.

Older subjects. No dose adjustment by age is proposed. Exposure did not differ materially with age in the population pharmacokinetic analysis. Dose reduction and discontinuation for an adverse reaction were more frequent in subjects aged 75 years and

above than in younger subjects, and closer haematological monitoring is advised in that group.

Hepatic and renal impairment. No dose recommendation can be made in severe hepatic or severe renal impairment, because subjects with severe impairment were excluded from the clinical programme. No dose adjustment is proposed for mild or moderate impairment.

4.3 Contraindications

Hypersensitivity to kelvaparib or to any of the excipients. Breast-feeding.

4.4 Special warnings and precautions for use

Myelosuppression

Myelosuppression is an important identified risk and is the most frequent reason for dose modification. In study KLV-7701 anaemia was reported in 202 of 418 subjects (48.3 per cent), thrombocytopenia in 101 subjects (24.2 per cent), neutropenia in 83 subjects (19.9 per cent) and leukopenia in 44 subjects (10.5 per cent).

Treatment should not begin until haematological recovery from prior chemotherapy is complete. Full blood count should be measured before the first dose, weekly for the first month and monthly thereafter. Treatment should be withheld for a grade 3 or higher cytopenia and resumed at a reduced dose on recovery. Where a cytopenia has not recovered within 28 days, haematological consultation and bone marrow examination should be considered.

Myelodysplastic syndrome and acute myeloid leukaemia

Myelodysplastic syndrome and acute myeloid leukaemia are an important identified risk and have been observed in the clinical programme, in 9 subjects (1.4 per cent) in study KLV-7701. Both have a latency substantially longer than the duration of exposure in the completed studies, so the cumulative incidence is expected to rise with further follow-up and should not be read as final.

A cytopenia that persists beyond 28 days, or a blood count abnormality that is disproportionate to recent exposure, should prompt bone marrow examination and cytogenetic analysis rather than continued dose modification. Where myelodysplastic syndrome or acute myeloid leukaemia is confirmed, treatment should be discontinued permanently and the subject referred for haematological management. Long-term follow-up for these events continues for five years after the last dose irrespective of the reason for discontinuation.

Pneumonitis

Pneumonitis is an important identified risk. New or worsening respiratory symptoms, including cough, dyspnoea or fever, should prompt interruption of treatment and

radiological investigation. Cough was reported in 55 of 418 subjects (13.2 per cent). Treatment should be discontinued permanently where pneumonitis is confirmed and no alternative cause is identified.

4.5 Interaction with other medicinal products and other forms of interaction

Kelvaparib is metabolised principally by cytochrome P450 3A4. Concomitant use with a strong inhibitor of that enzyme should be avoided; where unavoidable, a dose reduction to 150 mg twice daily is proposed. Concomitant use with a strong inducer should be avoided. The coding dictionary version used for concomitant medication in the clinical programme is recorded in the statistical documentation for each study.

4.6 Fertility, pregnancy and lactation

Kelvaparib may cause foetal harm. Women of childbearing potential should use effective contraception during treatment and for six months after the last dose. Breast-feeding is contraindicated during treatment and for one month after the last dose. An effect on fertility cannot be excluded and is listed as an important potential risk.

4.7 Effects on ability to drive and use machines

Fatigue and asthenia are very common and may affect the ability to drive or use machines.

4.8 Undesirable effects

The safety profile is based on 418 subjects who received kelvaparib 300 mg in study KLV-7701. Events of grade 3 or higher were reported in 298 subjects (47.5 per cent) and serious events in 159 subjects (25.4 per cent). Nausea was the most frequent event.

Frequencies are defined as: very common (at least 1 in 10), common (at least 1 in 100 to less than 1 in 10). Within each frequency grouping, adverse reactions are presented in order of decreasing incidence.

System organ class / Adverse reaction	Frequency	Incidence
Gastrointestinal disorders		
Nausea	Very common	61.5%
Vomiting	Very common	27.0%
Constipation	Very common	25.1%
Abdominal pain	Very common	20.1%
Diarrhoea	Very common	19.4%
Dyspepsia	Very common	10.5%
Blood and lymphatic system disorders		
Anaemia	Very common	48.3%
Thrombocytopenia	Very common	24.2%
Neutropenia	Very common	19.9%
Leukopenia	Very common	10.5%
General disorders		
Fatigue	Very common	47.4%
Asthenia	Very common	12.9%
Metabolism and nutrition disorders		
Decreased appetite	Very common	22.7%
Respiratory disorders		
Cough	Very common	13.2%
Psychiatric disorders		
Insomnia	Very common	11.0%
Infections and infestations		
Urinary tract infection	Common	9.3%

Incidence is the proportion of the 418 subjects treated with kelvaparib 300 mg in study KLV-7701 reporting the event at any grade, as presented in Table 12-2 of the clinical study report.
Myelodysplastic syndrome and acute myeloid leukaemia are not tabulated here because the observed cumulative incidence remains immature; they are described in section 4.4.

4.9 Overdose

There is no experience of overdose with kelvaparib. Management should be supportive and directed at the observed effects, with particular attention to haematological monitoring.

5. PHARMACOLOGICAL PROPERTIES

Kelvaparib is an orally administered selective inhibitor of poly (ADP-ribose) polymerase 1. Inhibition of that enzyme impairs single-strand break repair and is selectively lethal to tumour cells carrying a defect in homologous recombination repair. Exposure increases less than proportionally with dose across the range studied.

6. PROPOSED CHANGES UNDER CONSIDERATION

No change to this document is proposed at the date shown. The cumulative incidence of myelodysplastic syndrome and acute myeloid leukaemia is kept under review at each periodic reporting interval, and any proposal to strengthen the warning in section 4.4 will be documented with the supporting data, the wording proposed and the effective date.